In the Third Court of Appeals, Austin, Texas

CAUSE NO. 03-22-00126-CV

Greg Abbott, et al.,
Appellant,

v.

Jane Doe, et al.,
Appellee.

On Appeal from the 201st Judicial District Court, Travis County, Texas
Trial Court Cause No. D-1-GN-22-000977

BRIEF OF AMICUS CURIAE TEXAS MEDICAL ASSOCIATION

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IDENTITY OF PARTIES AND COUNSEL

Appellants

Greg Abbott, sued in his official capacity as Governor of the State of Texas; Jaime Masters, sued in her official capacity as Commissioner of the Texas Department of Family and Protective Services; and the Texas Department of Family and Protective Services.

Counsel for Appellants: Courtney Corbello (Texas State Bar No. 24097533), Ryan G. Kercher (Texas State Bar No. 24060998), Office of the Attorney General, P.O. Box 12548, Capitol Station, Austin, Texas 78711-2548, courtney.corbello@oag.texas.gov, ryan.kercher@oag.texas.gov.

Appellees

Jane Doe, individually and as parent and next friend of Mary Doe, a minor; John Doe, individually and as parent and next friend of Mary Doe, a minor; and Dr. Megan Mooney.

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Additional amicus curiae in support of the injunction: Ron Beal, Professor Emeritus & Attorney at Law (Texas State Bar No. 24005041), 2530 Wooddale Circle, Waco, TX 76710.

INTEREST OF THE AMICUS CURIAE

The Texas Medical Association ("TMA") is a private, voluntary, non-profit association of more than 55,000 Texas physicians and medical students. TMA was founded in 1853 to serve the people of Texas in matters of medical care, prevention and cure of disease, and improvement of public health. Today, TMA's maxim continues in the same direction: Physicians caring for Texans. TMA's diverse

physician members practice in all fields of medical specialization. TMA supports Texas physicians by providing distinctive solutions to the challenges they encounter in the care of patients.

TMA has a strong interest in this case, as demonstrated by TMA's policies. TMA policies: (1) recognize that transgender individuals have unique health care needs and suffer significant barriers in access to care that result in health care disparities, (2) oppose discrimination based on gender identity, and (3) oppose efforts to criminalize evidence-based, gender-affirming care for transgender youth. TMA's policies are adopted by TMA's House of Delegates—physicians who represent members' interests statewide. These policies, in relevant part, are set forth below for reference:

1. Policy No. 55.058 Sexual Orientation Change Efforts and Gender-Affirmation Therapies for Minors. (1) The Texas Medical Association supports treatment and therapies rooted in acceptance and support regarding an individual's sexual orientation and gender identification and therefore opposes practices aimed at changing an individual's sexual orientation, including conversion therapy; (2) TMA supports physician efforts to provide medically appropriate therapies relating to gender identity and opposes the criminalization of these practices; (3) TMA supports the prohibition of any person licensed to provide mental health counseling from engaging in sexual orientation change efforts with

patients younger than 18 years of age. TMA supports the practice of evidence-based therapies and will aggressively oppose the use of potentially harmful, unproven therapies for children.

2. Policy No. 55.066 Opposition to Criminalization of Gender-Affirming Care for Transgender Youth. The Texas Medical Association opposes efforts to criminalize evidence-based, gender-affirming care for transgender youth.
3. Policy No. 60.008 Rejection of Discrimination. The Texas Medical Association does not discriminate, and opposes discrimination, based on race, religion, disability, ethnic origin, national origin, age, sexual orientation, sex, or gender identity. TMA supports physician efforts to encourage that the nondiscrimination policies in their practices, medical schools, hospitals, and clinics be broadened to include "race, religion, disability, ethnic origin, national origin, age, sexual orientation, sex, or gender identity" in relation to patients, health care workers, and employees.
4. Policy No. 60.010 Opposing Legislation that Mandates Physician Discrimination. The Texas Medical Association… opposes legislation or regulation that mandates physicians and other health professionals discriminate against or limit access to health care for a specific patient population.
5. Policy No. 265.028 Improving LGBTQ Health Care Access. The Texas Medical Association recognizes that lesbian, gay, bisexual, transgender, queer, or

questioning (LGBTQ) individuals have unique health care needs and suffer significant barriers in access to care that result in health care disparities.

Pursuant to Rule 11 of the Texas Rules of Appellate Procedure, TMA confirms that is has received no compensation or fees in connection with the preparation or submission of this Brief and will be responsible for all attorney fees incurred in filing this Brief.

ISSUES PRESENTED

At issue is the trial court's grant of a plaintiffs/appellees' Application for Temporary Injunction, as well as the trial court's denial of defendants/appellants' Plea to the Jurisdiction. The validity of these actions in turn depends on the validity and enforcement of the Governor's letter and the DFPS statement, both issued February 22, 2022, as well as Attorney General Paxton's Opinion No. KP-0401, which they reference and incorporate, and whether they violate the constitutional rights of the appellees/plaintiffs and other persons, the Texas Administrative Procedure Act, Texas Gov't Code § 2001.038, or any other applicable laws.

STATEMENT OF FACTS

On August 23, 2021, Texas Representative Matt Krause submitted an opinion request to the Texas Office of the Attorney General on two issues related to the care of transgender youth. First, whether certain surgical sex-change

procedures, if performed on minors, constitute "abuse" under Chapter 261 of the Texas Family Code. Second, whether the administration of puberty-blockers or hormone therapy, as part of gender-affirming care of a minor, also constitutes "abuse" under Chapter 261.

On September 23, 2021, TMA and the Texas Pediatric Society submitted a brief in response to the request.[1] The brief limited its response to gender-affirming medical care—i.e., puberty blockers and hormone therapy—and did not address the issue of surgical care. The brief stated TMA's opposition to the criminalization of evidence-based, gender-affirming care for transgender youth and adolescents. The brief also explained that Representative's Krause's request was based on the premise of gender-affirming care being medically unnecessary, but evidence recognizes such care as medically necessary and appropriate.

On February 18, 2022, the Texas Office of the Attorney General released Opinion No. KP-0401, concluding that gender-affirming treatment of transgender children could legally constitute child abuse under Chapter 261 of the Texas Family Code.

Based on this opinion, on February 22, 2022, Texas Governor Greg Abbott sent a letter to the Commissioner of the Texas Department of Family and

[1]The September 23, 2021 letter brief is included in the Appendix as Exhibit A.

Protective Services (DFPS), Jaime Masters.[2] The Governor's letter directed the agency to investigate any reported instances of gender-affirming care of minors. The letter also noted that "Texas law imposes reporting requirements upon all licensed professionals who have direct contact with children who may be subject to such abuse, including doctors, nurses, and teachers, and provides criminal penalties for failure to report such child abuse."

On February 22, 2022, DFPS released a statement that the agency would comply with the Attorney General's opinion and Governor Abbott's letter and investigate reports of gender-affirming treatment of minors.

Appellee/plaintiff Jane Doe is the parent of Mary Doe, a transgender 16-year-old receiving gender-affirming care. Jane Doe is also a DFPS employee. On February 23, Jane Doe inquired with her direct supervisor about the impact of the Attorney General's opinion and Governor Abbott's letter on DFPS policy. Later that day, Jane Doe was placed on paid leave, and the following day was contacted by a Child Protective Services (CPS) investigator and informed that her family would be investigated for Mary Doe's gender dysphoria treatments.

On March 1, 2022, Jane Doe and her husband John Doe, individually and as parents of Mary Doe, filed suit against Governor Abbott, Commissioner Masters,

[2]Governor Abbott's February 22, 2022 letter, which contains the Attorney General's February 18, 2022 opinion, is included in the Appendix as Exhibit B.

and DFPS in Travis County District Court. Dr. Megan Mooney, PhD, a Houston psychologist who provides mental health evaluation for youth with gender dysphoria, was also named as a plaintiff in the lawsuit. The suit requested three avenues of relief:

1. A temporary restraining order (TRO) to preserve the status quo and restrain the defendants from investigating families based on the Governor's letter or the Attorney General's opinion;
2. A temporary injunction, followed by a permanent injunction, prohibiting the defendants from enforcing the Governor's letter, the Attorney General's opinion, or the DFPS statement, including the required reporting of children receiving gender-affirming care as suspected child abuse and the investigation of families of children being treated for gender dysphoria; and
3. Declaratory judgment that the DFPS statement is ultra vires, unconstitutional, and violates the Texas Administrative Procedures Act, and that the Governor's letter is also ultra vires and unconstitutional.

On March 2, 2022, the Honorable Judge Amy Clark Meachum issued an order granting the appellee/plaintiffs' application for a temporary restraining order, enjoining the defendants/appellants from taking any action against the plaintiffs based on the Attorney General's opinion, the Governor's letter, or the DFPS statement. Judge Meachum's order also set a March 11, 2022, hearing date for the

appellees/plaintiffs' request for a temporary injunction for potential statewide application.

Defendants/appellants immediately appealed to the Third Court of Appeal in Austin, on the basis that Judge Meachum's order implicitly denied the defendants' plea to the jurisdiction and seeking an accelerated interlocutory appeal of that issue (Case No. 03-22-00107-CV). On March 9, 2022, this Court dismissed the appeal for lack of jurisdiction.

On March 11, 2022, the plaintiffs/appellee's application for a temporary injunction was heard before Judge Meachum. That afternoon, Judge Meachum ordered that application granted, finding that there was a substantial likelihood that the plaintiffs/appellants would prevail after a trial on the merit due to the Governor's letter being ultra vires, beyond the scope of his authority, and unconstitutional, and the DFPS statement is similarly void. The court enjoined the appellants/defendants from enforcing the Governor's letter, the DFPS statement, and the Attorney General's opinion:

> This Temporary Injunction restrains the following actions by the Defendants: (1) taking any actions against Plaintiffs based on the Governor's directive and DFPS rule, both issued February 22, 2022, as well as Attorney General Paxton's Opinion No. KP-0401 which they reference and incorporate; (2) investigating reports in the State of Texas against any and all persons based solely on alleged child abuse by persons, providers or organizations in facilitating or providing gender-affirming care to transgender minors where the only grounds for the purported abuse or neglect are either the facilitation or provision of gender-affirming medical treatment or the fact that the

> minors are transgender, gender transitioning, or receiving or being prescribed gender-affirming medical treatment; (3) prosecuting or referring for prosecution such reports; and (4) imposing reporting requirements on persons in the State of Texas who are aware of others who facilitate or provide gender-affirming care to transgender minors solely based on the fact that the minors are transgender, gender transitioning, or receiving or being prescribed gender-affirming medical treatment.

Judge Meachum also issued an order denying defendants/appellants' Plea to the Jurisdiction and set the trial on the merits for July 11, 2022.[3] That same evening, appellants/defendants filed Notice of Accelerated Interlocutory Appeal, appealing the denial of their Plea to the Jurisdiction and the granting of plaintiffs/ appellees' Request for Temporary Injunction.

SUMMARY OF THE ARGUMENT

This brief addresses the acceptance and medically necessity of gender-affirming medical care for adolescents with gender dysphoria, and the beneficial results of such treatment. The brief also addresses TMA's concern that the recent state actions will create barriers in access to care for transgender adolescents, a vulnerable population that already faces barriers in access to care.

[3] Judge Meachum's March 11, 2022 Orders are included in the Appendix as Exhibit C. The defendant/appellants' March 11, 2022 Notice of Accelerated Interlocutory Appeal is included in the Appendix as Exhibit D.

ARGUMENT

I. Gender-Affirming Care is Accepted and Medically Necessary Treatment for Adolescents with Gender Dysphoria

Providing gender-affirming care is consistent with accepted clinical standards for the treatment of adolescents with gender dysphoria. This treatment should not be criminalized or stigmatized.

For transgender adolescents with gender dysphoria, treatment with puberty-blockers and hormone therapy is consistent with the treatment guidelines from the World Professional Association for Transgender Health[4] and the Endocrine Society.[5] The latter guidelines are co-sponsored by the Pediatric Endocrine Society.[6] As set forth below, these standards have been recognized and endorsed in the clinical guidance and publications of every American medical association that has addressed this area, as well as the in guidelines of the American Psychological Association.

> The American Academy of Pediatrics:
>
> Gender affirmation among adolescents with gender dysphoria often reduces the emphasis on gender in their lives, allowing them to attend

[4]World Professional Association for Transgender Health. Standards of Care for the Health of Transsexual, Transgender and Gender Nonconforming People, 7th Version (2012).

[5]Hembree W, et al. Endocrine Treatment of Gender-Dysphoric/Gender-Incongruent Persons: An Endocrine Society Clinical Practice Guideline. THE JOURNAL OF CLINICAL ENDOCRINOLOGY & METABOLISM, Volume 102, Issue 11, pages 3869–3903 (Nov. 2017).

[6]Id. at 3869.

> to other developmental tasks, such as academic success, relationship building, and future-oriented planning. Most protocols for gender-affirming interventions incorporate World Professional Association of Transgender Health and Endocrine Society recommendations.[7]

The American College of Obstetrics and Gynecology:

> [H]ormone therapy is a medically necessary treatment for many transgender individuals with gender dysphoria.[8]

For more details on the provision of hormone therapy for these populations, obstetrician–gynecologists should see resources from the World Professional Association for Transgender Health and the Endocrine Society.[9]

The American College of Physicians:

> The Endocrine Society and WPATH provide straightforward approaches to medical care for transgender patients. For children and adolescents, [these] organizations promote a multidisciplinary approach that includes both mental health professionals for assessment and medical professionals for medical interventions.[10]

The American Psychological Association:

> The [psychological] Guidelines are intended to complement treatment guidelines for TGNC people seeking mental health services, such as

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[7]Rafferty J. Ensuring Comprehensive Care and Support for Transgender and Gender-Diverse Children and Adolescents. American Academy of Pediatrics, Committee on Psychosocial Aspects of Child and Family Health, Committee on Adolescence and Section on Gay, Lesbian, Bisexual and Transgender Health and Wellness. PEDIATRICS, Vol. 142, No. 4, page 6 (Oct. 2018).

[8]American College of Obstetricians and Gynecologists, Committee on Gynecologic Practice and Committee on Health Care for Underserved Women Health Care for Transgender and Gender Diverse Individuals. Committee Opinion No. 823, page 75 (Mar. 2021).

[9]Id. at 81.

[10]Safer J, Tangpricha V. Care of the Transgender Patient. ANNALS OF INTERNAL MEDICINE, page 4 (Jul. 2019).

> those set forth by the World Professional Association for Transgender Health Standards of Care and the Endocrine Society.[11]
>
> [T]here is greater consensus that treatment approaches for adolescents affirm an adolescents' gender identity. Treatment options for adolescents extend beyond social approaches to include medical approaches. One particular medical intervention involves the use of puberty-suppressing medication or "blockers" (GnRH analogue), which is a reversible medical intervention used to delay puberty for appropriately screened adolescents with gender dysphoria.[12]
>
> Hormone therapy (gender-affirming hormone therapy, hormone replacement therapy): the use of hormones to masculinize or feminize a person's body to better align that person's physical characteristics with their gender identity…. Hormone therapy may be an important part of medically necessary treatment to alleviate gender dysphoria.[13]
>
> Puberty suppression (puberty blocking, puberty delaying therapy): a treatment that can be used to temporarily suppress the development of secondary sex characteristics that occur during puberty in youth, typically using gonadotropin-releasing hormone (GnRH) analogues. Puberty suppression may be an important part of medically necessary treatment to alleviate gender dysphoria. Puberty suppression can provide adolescents time to determine whether they desire less reversible medical intervention and can serve as a diagnostic tool to determine if further medical intervention is warranted.[14]

Additionally, in 2020 the American Psychiatric Association affirmed its support for access to "affirming and supportive treatment for trans and gender

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[11] American Psychological Association. Guidelines for Psychological Practice with Transgender and Gender Nonconforming People. AMERICAN PSYCHOLOGIST. Vol. 70, No. 9, page 833 (Dec. 2015).

[12] Id. at 842.

[13] Id. at 862.

[14] Id.

diverse youth and their families," including puberty blockers and hormone therapy.[15]

The Attorney General's opinion characterizes gender-affirming medical treatments as "elective" or "not medically necessary." However, the opinion does not provide a basis for this characterization.[16] Generally, the determination of medical necessity of a particular treatment is made on an individual basis by the patient's physician, as part of the practice of medicine.[17] Whether or not a proposed treatment is medically necessary should be decided in a manner consistent with generally accepted standards of medical practice that a prudent physician would provide to a patient for the purposes of preventing, diagnosing or treating an illness, injury, disease or its symptoms.[18]

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[15] American Psychiatric Association. Position Statement on Treatment of Transgender (Trans) and Gender Diverse Youth (Jul. 2020).

[16] The Office of the Attorney General has previously disclaimed the ability to determine medical necessity in an Attorney General opinion. See Tex. Atty. Gen. Op. No. JM-746 (1987) ("The determination of what specific services are medically necessary is a question of fact and cannot be resolved in the opinion process.").

[17] TMA Board of Councilors' Opinions, Medical Necessity ("The determination of medical necessity is the practice of medicine."); see also Tex. Occ. Code § 151.002(13) ("'Practicing medicine' means the diagnosis, treatment, or offer to treat a mental or physical disease or disorder.").

[18] TMA Board of Councilors' Opinions, Medical Necessity; see also AMA Health Policy H-320-953, Definitions of "Screening" and "Medical Necessity" ("Our AMA defines medical necessity as: Health care services or products that a prudent physician would provide to a patient for the purpose of preventing, diagnosing or treating an illness, injury, disease or its symptoms in a manner that is… in accordance with generally accepted standards of medical practice."); Tex. Civ.

As illustrated above, providing gender-affirming care is consistent with accepted clinical standards for the treatment of adolescents with gender dysphoria. These beneficial and potentially life-saving treatments should not be indiscriminately characterized as "not medically necessary."

II. Banning Gender-Affirming Care Will Create More Barriers in Access to Care for Transgender Adolescents

TMA recognizes that transgender individuals have unique health care needs and suffer significant barriers in access to care, which result in health care disparities. The state's recent actions will likely create additional barriers, worsening these disparities.

As a direct result of the state's recent actions, transgender adolescents will likely be unable to initiate or continue gender-affirming treatments.[19] Gender-affirming care is provided to reduce distress and prevent harm. Transgender youth and adolescents are particularly at risk for depression and suicidal ideations.[20]

Prac. & Rem. Code Ann. § 88.001(1) ("'Appropriate and medically necessary' means the standard for health care services as determined by physicians and health care providers in accordance with the prevailing practices and standards of the medical profession and community.").

[19]The recent state actions have already begun to prevent transgender adolescents' access to gender-affirming care by in Texas. In response to Attorney General's opinion and Governor Abbott's letter, Texas Children's Hospital announced that it had paused hormone therapies for gender-affirming treatment. See Dey, Sneha. Houston hospital pauses hormone therapy for transgender children as threats of child abuse investigations loom. TEXAS TRIBUNE (Mar. 4, 2022).

[20]Rafferty J. Ensuring Comprehensive Care and Support for Transgender and Gender-Diverse Children and Adolescents, page 3 ("In 1 retrospective cohort

When transgender adolescents are provided with appropriate gender-affirming care though, depression and suicidal ideation decreases.[21] However, preventing or halting this treatment has the potential to worsen a transgender adolescent's gender dysphoria.[22]

The state's recent actions may also dissuade transgender adolescents from accessing medical care unrelated to their transgender status. Physicians are among the professionals required to report abuse under Chapter 261.[23] Transgender adolescents may fear that a visit to a physician could reveal that they have received

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study, 56% of youth who identified as transgender reported previous suicidal ideation, and 31% reported a previous suicide attempt.").

[21] Turban JL, et al. Pubertal Suppression for Transgender Youth and Risk of Suicidal Ideation. PEDIATRICS (Jan. 2020); see also American Psychiatric Association. Position Statement on Treatment of Transgender (Trans) and Gender Diverse Youth (Jul. 2020) ("Trans-affirming treatment, such as the use of puberty suppression, is associated with the relief of emotional distress, and notable gains in psychosocial and emotional development, in trans and gender diverse youth."); Rafferty J. Ensuring Comprehensive Care and Support for Transgender and Gender-Diverse Children and Adolescents, page 5 ("The available data reveal that pubertal suppression in children who identify as TGD generally leads to improved psychological functioning in adolescence and young adulthood."); Tordoff D, et al. Mental Health Outcomes in Transgender and Nonbinary Youths Receiving Gender-Affirming Care. JAMA Netw Open, page 2 (Feb. 2022) ("This study found that gender-affirming medical interventions were associated with lower odds of depression and suicidality over 12 months.").

[22] See American Psychiatric Association. Position Statement on Treatment of Transgender (Trans) and Gender Diverse Youth (Jul. 2020) ("Due to the dynamic nature of puberty development, lack of gender-affirming interventions… is not a neutral decision; youth often experience worsening dysphoria and negative impact on mental health as the incongruent and unwanted puberty progresses..").

[23] Tex. Family Code § 261.101.

gender-affirming treatment, triggering a DFPS investigation. They may therefore avoid necessary care.[24]

CONCLUSION AND PRAYER

For reasons above, Amicus Curiae Texas Medical Association respectfully requests that this Court upholds the trial court's temporary injunction.

[24]The Department of Health & Human Services, Office for Civil Rights has announced that reporting parents seeking gender-affirming care for their child to state authorities may constitute discrimination that violates Section 1557 of the Affordable Care Act. U.S. DEPARTMENT OF HEALTH AND HUMAN SERVICES, OFFICE FOR CIVIL RIGHTS. HHS Notice and Guidance on Gender Affirming Care, Civil Rights, and Patient Privacy (Mar. 2, 2022). Thus, in addition to creating an additional barrier to care for transgender adolescents, the required reporting places physicians in an untenable position too. Those who do not report gender-affirming treatment of adolescents may face possible state criminal actions being brought against them, while those who do report may face federal investigation.

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CERTIFICATE OF COMPLIANCE

I certify that this Brief complies with the type-volume limitation of Tex. R. App. P. 9.4 because it contains approximately 3,342 words, excluding the parts of the brief exempted by Tex. R. App. P. 9.4. I also certify that this Brief complies with the typeface requirements of Tex. R. App. P. 9.4(e) because it has been prepared in a proportionally spaced typeface using Microsoft Word 2016 in 14-point Times New Roman font and 12-point Times New Roman font for footnotes.

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I certify that on this 18th of March, 2022, a true and correct copy of the accompanying Brief of Amicus Curiae Texas Medical Association was electronically served on all counsel of record:

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